

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 17 (FINAL)

New Case: Rohan (26 y/o) – Contamination OCD + Checking

Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	17 of 17 (Final)	Focus:	Termination, Review & Long-Term Maintenance

PRE-SESSION THERAPIST NOTES

This is the final termination session. Client has made steady, realistic progress throughout the 17 sessions despite initial shame, exhaustion, and skepticism. He has developed strong, personalized tools and a clear relapse prevention / maintenance plan. Today we will do a comprehensive review of progress since Session 1, celebrate achievements, finalize the maintenance plan, provide emotional closure, and formally end the main course of therapy while confirming booster sessions.

SESSION START – OPENING & MOOD CHECK

Therapist: Good morning, Rohan. Today is our final session. How are you feeling as we begin?

Client: (speaking slowly, with a mix of pride and emotion) I feel proud that I kept coming even when it was very hard at the beginning. I also feel a bit sad that this is the last session, but I feel much more capable now than when we started. I still get nervous sometimes, but I know what to do.

Therapist: (speaking gently) That combination of pride, some sadness, and realistic confidence is very normal and healthy at this stage. We'll take time today to honor the work you've done and make sure you feel fully equipped moving forward.

COMPREHENSIVE PROGRESS REVIEW

Therapist: Let's take a moment to look back at where we started. When you first came in, you were washing your hands 25–40+ times a day, checking switches and locks multiple times every night, avoiding touching surfaces and social interactions, and feeling exhausted and ashamed. Where are things now?

Client: I'm washing much less — usually just normal washing now, sometimes a bit more when I'm stressed, but nothing like before. I still check switches sometimes, but much less often. I'm touching more things without immediately washing, and I'm delaying when I do feel the urge. I'm going out more and interacting with colleagues without as much fear. I still get anxious sometimes, but I know how to handle it now. I feel much less exhausted and ashamed.

Therapist: Those are meaningful and realistic changes. What do you think has been the single most important thing that helped create these changes?

Client: Doing the exposures gradually and seeing that the anxiety comes down by itself was huge. Also, labeling the thoughts as "OCD thoughts" helped me create distance. And having a clear Action Plan for what to do if things start getting worse makes me feel more secure.

FINAL RELAPSE PREVENTION & MAINTENANCE REVIEW

Therapist: Let's do one final review of your complete maintenance plan.

Early Warning Signs: Increased washing or checking, stronger urges, more avoidance of situations or people, increased irritability and fatigue, more frequent contamination thoughts that feel harder to manage.

Top Coping Strategies: Labeling ("This is an OCD thought"), urge surfing, diaphragmatic breathing, grounding techniques, reviewing values and long-term goals, practicing relevant exposures from the hierarchy.

Action Plan if symptoms return: Increase use of labeling and coping skills immediately → Practice one relevant exposure daily for 1 week → Increase monitoring for 1 week → If symptoms do not improve after 7–10 days or significantly affect daily life → Contact therapist for a booster session.

Long-term Maintenance: Continue practicing one exposure from the hierarchy occasionally (even on good days) to keep skills sharp. Review Action Plan weekly for the first 1–2 months, then monthly. Stay connected to values (connection, energy, self-acceptance).

Booster Sessions: One at 6 weeks and one at 3 months after today (or earlier if needed).

Therapist: This is a very complete and personalized plan. How confident do you feel about using it?

Client: I feel confident. I know what to watch for and exactly what to do if I need to. I still get nervous sometimes, but I know I have tools now.

EMOTIONAL CLOSURE

Therapist: As we come to the end of our work together, is there anything you want to say or ask?

Client: I just want to say thank you. At the beginning, I felt ashamed and thought I should be able to fix this myself. You helped me understand that OCD is not about willpower, and you never judged me. I feel much more capable now. I still have work to do, but I know I can do it.

Therapist: Thank you, Rohan. It has been a privilege to work with you. You did the hard work — showing up every week even when it was exhausting, being honest, practicing between sessions, and facing your fears gradually. The changes you've made are because of your effort. I'm very proud of you.

Client: That means a lot. I'll miss this, but I'm also ready.

FINAL WORDS & BOOSTER CONFIRMATION

Therapist: Before we end, let's confirm your booster sessions:

- **Booster Session 1**: 6 weeks from today
- **Booster Session 2**: 3 months from today

Therapist: You can always reach out earlier if needed. You now have everything you need to continue growing. I have complete confidence in you.

Client: Thank you. I'll see you in 6 weeks for the first booster.

POST-SESSION THERAPIST NOTES

- Excellent, stable outcome after 17 sessions despite initial shame, exhaustion, and skepticism.
- Client has developed strong, personalized, and sustainable coping skills for OCD.
- Significant reduction in washing, checking, and avoidance. Client reports increased confidence and self-compassion.
- Comprehensive relapse prevention / maintenance plan is clear, realistic, and personalized.
- Booster sessions confirmed (6 weeks + 3 months).
- Therapy successfully terminated with warmth, closure, and confidence in the client's long-term success.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.